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Case Number: 24SMCV05452 Hearing Date: July 14, 2026 Dept: P

Tentative

Ruling

Ryden

L. Loong v. Edmund G. Sarraf, M.D., et al., Case No. 24SMCV05452

Unilab

Corporation's Motion for Leave to Conduct Two additional physical medical examinations.

Hearing

Date: July 14, 2026

Background

Plaintiff

Ryden L. Loong, a minor, by and through his guardian ad litem, Sepideh Loong, sues Defendants Edmund G. Sarraf, M.D.; Unilab Corporation d/b/a Quest Diagnostics Clinical Laboratories, Inc. ("Quest"); The Southern California Integrative Wellness Center; The Regents of the University of California ("The Regents"); and UCLA Medical Center for medical malpractice. Loong alleges he developed bilateral sensorineural hearing loss and mobility impairments as a result of Auditory Neuropathy Spectrum Disorder (ANS), caused by hyperbilirubinemia due to Quest's errors processing his blood tests and the other Defendants' delays in providing appropriate treatment. (Complaint, ¶

On

November 7, 2024, Loong filed his complaint.

On

February 6, 2025 Quest, Sarraf, and Southern California Integrative Wellness Center filed their answers. On February 24, 2025, The Regents answered.

On

February 24, 2025, Loong dismissed UCLA Medical Center on the grounds that it is an entity subsumed by The Regents.

On January

8, 2026, the parties filed a stipulated protective order.

On May 20,

2026, The Regents filed a motion for summary judgment.

On May 21,

2026, the parties stipulated to continue trial and all corresponding deadlines to September 14, 2026.

On June 4,

2026, Quest filed this motion for leave to conduct two additional physical examinations: a genetics examination and a neurotology exam with an auditory brainstem response test. On June 5, 2026, The Regents joined Quest's motion. On June 30, 2026, Plaintiffs filed opposition. On July 7, 2026, Quest replied.

Arguments

Quest

moves for a court order authorizing two independent medical examinations in addition to the neurological independent medical examination the parties stipulated to. Quest argues that good cause exists for a genetic examination because Loong's medical records show that he was born with microcephaly and a G6PD deficiency, a hereditary enzymatic defect that may have contributed to or caused Loong's hearing loss. (Id. at p. 9.) Quest argues that a genetic examination is necessary to determine if Loong's hearing loss could have a genetic factor. (Id. at p. 10.) Quest then argues that good cause also exists for an auditory brainstem response examination to confirm Loong's diagnosis and evaluate his damages. (Id. at p. 11.)

In

opposition, Loong argues that Quest does not demonstrate good cause in light of Plaintiff's statutory and constitutional privacy rights. (Opposition, p. 4.)

Loong argues that Quest's experts have not properly laid the foundation for their opinions. (Id. at p. 6.) Loong also argues that the "possibility" of an underlying genetic condition is insufficient to justify an additional medical exam. (Ibid.) Loong argues that Quest's request for a genetic test is too vague and opens the door for the defense to collect protected genetic material for unspecified testing. (Id. at p. 7.) Loong argues that the identified indicators for genetic testing do not necessarily lead to the conclusion that there is a genetic issue—Loong's microcephaly appeared to resolve after birth, and his G6PD deficiency is very common and does not always cause hearing loss. (Id. at p. 8). Loong also argues that a G6PD deficiency is incredibly common, and most effected individuals have not been diagnosed with ANSD. (Id. at p. 10.) Finally, Loong argues that all of the tests that Quest requests have already been conducted by others fairly recently. (Id. at p. 11.)

In reply,

Quest argues that Loong's arguments confirm that these issues are factual and should be resolved through cross-examination at trial. (Reply, p. 3.) Quest argues that hyperbilirubinemia is not the only recognized cause of hearing loss, and that other conditions, many genetic, account for 40% of cases. (Id. at p. 4.) Quest argues that it will preserve Loong's privacy and that Loong has placed his genetic status in controversy because his G6PD deficiency is undisputedly hereditary. (Id. at p. 5.) Quest argues that the eggshell plaintiff doctrine is irrelevant and that the neurologic examination is not duplicative. (Id. at pp. 7-8.)

Evidentiary

Objections

Loong

objects to the Declaration of Elina Kari, M.D. in its entirety, and specifically to ¶¶ 3-4 on the grounds that Dr. Kari does not properly lay the foundation for her medical opinion. The Court disagrees. Dr. Kari testified that she reviewed Loong's medical history and the record in this case and based her opinion on that record. (Kari Decl., ¶ 3.) Loong's objections are overruled.

Loong also

objects to the Declaration of Robin Clark, M.D., in its entirety and ¶¶ 3-6 specifically, on the same grounds. Loong's objections are overruled for the

same reason Loong's objections to the Kari Declaration are overruled. (See Clark Decl., ¶ 3.)

Motion to Conduct Medical Examination

"If any party desires to obtain discovery by a physical examination other than that described in Article 2 (commencing with Section 2032.210), or by a mental examination, the party shall obtain leave of court." (Code Civ. Proc., § 2032.310, subd. (a).) "A motion for an examination under subdivision (a) shall specify the time, place, manner, conditions, scope, and nature of the examination, as well as the identity and the specialty, if any, of the person or persons who will perform the examination. The motion shall be accompanied by a meet and confer declaration under Section 2016.040." (Id. subd. (b).)

"The court shall grant a motion for a physical or mental examination under Section 2032.310 only for good cause shown." (Code Civ. Proc., § 2032.320, subd. (a).) Section 2032.320 subdivision (b) requires a motion for an IME to specify "the time, place, manner, conditions, scope, and nature of the examination" (Code Civ. Proc., § 2032.310 subd. (b).) Moreover, An order granting a physical or mental examination shall specify the person or persons who may perform the examination, as well as the time, place, manner, diagnostic tests and procedures, conditions, scope, and nature of the examination." (Code Civ. Proc. § 2032.320, subd. (d).)

The California Supreme Court has held that a prior, but substantially similar, statute allowing for IMEs "defines a showing of 'good cause' as requiring that the party produce specific facts justifying discovery and that the inquiry be relevant to the subject matter of the action or reasonably calculated to lead to the discovery of admissible evidence." (Vinson v. Superior Court (1987) 43 Cal.3d 833, 840.)

Discussion

The Clark and Keri Declarations specify the time, place, manner, conditions, and scope of

the proposed examinations. Dr. Keri proposes to conduct a neurotologic history and physical examination; audiologic evaluation, including behavioral audiometry; ABR testing; and any indicated, non-invasive, audiologic or electrophysiologic measures with Dr. Zettner and Dr. Gibson for 2-4 hours at the University of California San Diego. (Keri Decl., ¶ 6.) Dr. Clark proposes a standard clinical genetics consultation by a board-certified geneticist: a 90 minute to 2 hour medical history, physical and dysmorphology examination, buccal swab for genetic testing, a review of growth records, and any other warranted testing. (Clark Decl., ¶ 6.) The proposed examination would take place at a time and place convenient to Loong.

a.

Good

Cause for the neurotology examination

Quest

argues that there is good cause for the neurotology examination for two reasons. First, to confirm Loong's ANSD diagnosis, and second, to evaluate the nature and extent of his damages. (Motion for leave to conduct two physical examinations, pp. 7-8.) Quest's expert declaration explains that ANSD "is a type of hearing impairment characterized by preserved outer hair cell function (often normal otoacoustic emissions) but abnormal or absent auditory brainstem response (ABR) and poor speech discrimination out of proportion to pure-tone thresholds." (Kari Decl., ¶ 4.) "A comprehensive neurotologic evaluation, including a detailed history, physical examination of the ears and cranial nerves, and targeted audiologic testing, is the accepted standard of care to confirm or rule out ANSD, quantify the degree and nature of any hearing loss, assess functional impact, and determine whether the condition is stable, progressive, or attributable to the neonatal hyperbilirubinemia alleged in this case." (Ibid.)

In

opposition, Loong only argues that these tests would be duplicative of testing and ABR assessments Loong has undergone at UCLA Medical Center, USC Caruso Family Center for Childhood Communication, Project TALK, and Cedars Sinai Medical Group ENT. (Opposition, p. 4.)

Here, Loong

does not dispute that the proposed tests are the standard of care for diagnosing and determining the nature and extent of hearing loss in patients like

Loong. In fact, Loong admits that he has undergone extensive evaluation of this type for precisely this purpose. Loong's complaint is based on his ANSD diagnosis and resulting long-term hearing loss, placing those conditions directly at issue. This type of testing was necessary to make Loong's initial diagnosis, which is the subject of this litigation. Quest has good cause to evaluate those claims with their own expert examinations.

b.

Good

Cause for the Genetic Examination

Quest

contends that there is good cause for the genetic examination because Loong was born with microcephaly and a G6PD deficiency, an "X-linked genetic disorder."

Quest's expert evidence cites that G6PD deficiency can lead to hyperbilirubinemia and Loong's hearing loss when left untreated. (Clark Decl., ¶ 4.) Specifically, "the presence of microcephaly and SGA at birth raises the distinct possibility of an underlying genetic syndrome, chromosomal microdeletion/duplication, or other constitutional genetic condition that may independently contribute to, or interact with, the hyperbilirubinemia and the claimed long-term neurological injuries." (Ibid.)

Loong

correctly points out that he has a constitutional right to genetic privacy that requires a proposed genetic exam to be directly relevant to a claim or defense and essential to a claim's fair resolution. (Vinson v. Superior Court (1987) 43 Cal.3d 833, 842.) Loong also argues convincingly that his microcephaly resolved soon after birth and that the G6PD deficiency is incredibly common.[1] The Court is particularly concerned about the extent to which the proposed genetic test produces private information that is not relevant to this case in addition to information that is relevant. Though Quest describes the genetic test as minimally stressful, it is not clear if the genetic test they intend to run is a type of test that would reveal substantially more about Loong's genetic background than is relevant to his hearing loss.[2]

Dr.

Clark's supplemental declaration, however, explains in great detail why this genetic testing is indicated: Loong's MRI showed abnormalities in brain regions that are not typical sites of injury from hyperbilirubinemia. (Clark Decl., ¶ 17.)

Dr. Clark also explains that these tests are the standard of care to evaluate

potential genetic conditions in a child with Loong's symptoms and are "directly responsive" to Loong's microcephaly, SGA, G6PD, ANSD, and seizures. (Id. at p. 19.) Though this still does not provide additional information about the scope of the testing, it provides very strong evidence that there is good cause for the proposed genetic tests in this case. As such, the Court finds that Quest has shown good cause for genetic testing, but would like to discuss with the parties the scope of the proposed testing to ensure that the testing is limited to conditions which may be relevant to the injuries alleged.

Conclusion

Quests

Motion for Leave to Conduct Two Independent Medical Examinations is GRANTED.

[1]

Loong does not submit expert declarations to support this argument, however.

[2]

For example, some types of genetic tests show things like BRCA1 or BRCA2 gene mutations, which are not relevant to this case at all.